

Chapter 4 (Revised): Vermont as Policy Laboratory

Vermont as Policy Laboratory: Acts 167 and 68 and the Architecture of Mandated Transformation

Source integration: Act 167 (2022), Act 68 (2025), Oliver Wyman Act 167 Community Engagement: Recommendations (September 2024, revised October 2024), GMCB February 2026 RBP/Global Budget Report, Vermont Rural Health Transformation Program Application (November 2025), AHS January 2025 Legislative Presentation.

The Anatomy of a System-Level Reform Mandate

Most healthcare reform unfolds incrementally — a new demonstration model here, a revised billing rule there — accumulating over years into changes that are difficult to attribute to any single decision. Vermont's reform trajectory between 2022 and 2025 is different in kind, not degree. Within three years, the Vermont General Assembly enacted a sequence of legislation that collectively mandated the most comprehensive statewide healthcare restructuring attempted by any U.S. state since Maryland's all-payer model in the 1970s. Understanding this sequence — its logic, its mechanisms, and its unresolved tensions — is essential preparation for any healthcare leader operating in an environment of payment and delivery system transformation.

The sequence has a clear internal logic. Act 167 of 2022 was a diagnostic mandate: commission a rigorous, public, community-grounded diagnosis of what is actually wrong. Act 51 of 2023 was a planning mandate: begin doing something about it. Act 68 of 2025 was an operational mandate: specific tools — reference-based pricing, global hospital budgets, a Statewide Health Care Delivery Strategic Plan — were authorized and given statutory deadlines with real consequences for non-compliance. Each act built on the evidence and institutional capacity developed by its predecessor. The whole sequence constitutes what policy scholars call a reform cascade: a legislatively structured process in which each intervention creates the conditions for the next.

This chapter section examines each act, the analytical infrastructure it created, and the six-pillar implications for organizations navigating comparable reform environments. It draws heavily on the Oliver Wyman Act 167 Community Engagement: Recommendations report — the most comprehensive independent public diagnosis of a state hospital system in recent American health policy history — which must be read as the intellectual foundation for everything Vermont's legislature did in 2025 and everything AHS is mandated to deliver by 2028.

The Three Imperatives: Oliver Wyman's Central Finding

Before examining the legislative sequence, it is worth stating plainly what the Oliver Wyman analysis concluded at the highest level of abstraction. After 230 meetings across Vermont's 14 Hospital Service Areas, engagement with more than 3,100 participants representing over 100 organizations, and systematic analysis of financial, demographic, and quality data spanning every hospital in the state, Oliver Wyman's executive summary distilled Vermont's challenge to three imperatives:

Imperative 1: Build housing and other facilities and fix transportation.

Imperative 2: Pay PPS hospitals with reference-based pricing and move to global budgets/capitation for all when conditions for success are met.

Imperative 3: Move all care possible out of hospitals.

These three imperatives are not a wish list. They are the analytically derived conclusion of a year-long process that examined every dimension of Vermont's system. Each imperative maps directly onto the HTR six-pillar framework: the housing and transportation imperative is an Equity and Operations pillar challenge; the pricing and

global budget imperative is an Economics and Policy pillar challenge; and the shift of care out of hospitals is a Clinical pillar challenge that requires Technology and Operations pillar execution. The entire Vermont reform agenda can be read as a simultaneous, coordinated intervention across all six pillars — which is precisely why it is analytically instructive for any organization facing comparable pressures.

Oliver Wyman was equally direct about what will not work: more funding without structural change. Their analysis showed that simply increasing funding to attract more providers would drive up prices without improving access given Vermont's labor market constraints. Raising taxes or commercial premiums further risks driving residents and businesses out of the state. And the rural nature and shrinking population of most Vermont communities means that many areas simply cannot sustain a hospital with a full-time, full-spectrum of specialties. The implication is unambiguous: easy solutions — pour in money, add providers, maintain the status quo — will not resolve Vermont's healthcare crisis. The system must be structurally redesigned.

The Platform for Change: Vermont's System in Crisis

The Oliver Wyman report documented five interconnected challenge domains that constitute what it called the Platform for Change — the evidence base that makes transformation not just desirable but unavoidable. Each domain has precise, quantified data that the book's previous draft understated. The full picture is considerably more alarming than most public discussion of Vermont healthcare acknowledges.

Decreasing Affordability

Vermont's healthcare cost trajectory has been disconnecting from household income for at least six years, with no natural correction mechanism in sight. Oliver Wyman documented that average premiums for silver exchange plans reached \$985 in 2024 — a 108% price increase in six years. Individual and small group plan premiums increased 60-80% in the same period. Out-of-pocket maximums more than doubled in five years. Meanwhile, Vermont median household income grew only 22% between 2018 and 2022. Hospital approved charge increases grew 38% over the same period, compared to income growth of 22%. The gap is structural: hospital price growth has been running at two to three times income growth, and there is no market mechanism that closes this gap without regulatory intervention.

The premium data from Vermont Blue Cross Blue Shield confirms and extends this picture. Average annual per-member-per-month medical and pharmacy costs rose from \$481 in 2020 to \$812 in 2024 for local claims — a 69% increase in four years. Out-of-pocket maximums doubled in five years. A 2025 state survey found that many Vermonters, even those with insurance, delay care due to fears of medical debt. The affordability crisis is no longer a risk on the horizon; it is a current condition affecting clinical behavior.

Deteriorating Hospital Financial Sustainability

The financial projections in the Oliver Wyman report are the most important data in the entire Vermont reform story, and they need to be stated with the precision Oliver Wyman provided, not rounded down to reassuring generalities.

In FY2023, nine of Vermont's fourteen hospitals reported operating losses, with the worst reaching -8.9%. Oliver Wyman modeled two scenarios for the trajectory through 2028. Under the conservative scenario — 3.5% annual non-340B revenue growth and 5% annual expense growth — 13 of 14 hospitals are projected to report losses by 2028, with a cumulative 5-year system-wide deficit of \$700 million against break-even (\$1.4 billion against a 3% target operating margin). This is before accounting for the fact that Vermont hospitals collectively requested \$285 million in budget increases for FY25 versus FY24 alone, suggesting the conservative scenario may already be understated.

Under the more realistic scenario — accounting for 10% annual expense growth on non-physician labor, 7-8% growth on other operating expenses, and constrained revenue — the 5-year cumulative deficit reaches \$2.4 billion against break-even, or \$3.1 billion against a 3% operating margin. This is approximately \$3,700 to \$4,800 per

Vermont resident over five years. The UVM Health Network, which accounts for more than half the state's hospital capacity, faces \$1.5 billion in cumulative deficits over five years in the high-expense scenario.

These projections assume no structural change. They are not predictions of what will happen — they are the analytical case for why structural change is necessary. Without the interventions mandated by Acts 167 and 68, Vermont's hospital system does not gradually decline; it collapses, with hospitals closing unexpectedly rather than through orderly transformation.

Vermont in Practice: The Hospital Financial Crisis by the Numbers

Oliver Wyman's financial modeling established the quantitative case for urgent intervention. The key data points every healthcare leader and policymaker should know:

- 9 of 14 Vermont hospitals reported operating losses in FY2023, with losses reaching -8.9%
- Under conservative assumptions (3.5% revenue growth, 5% expense growth): 13 of 14 hospitals report losses by 2028; \$700M cumulative 5-year system deficit
- Under realistic assumptions (7-8% expense growth): \$2.4B cumulative 5-year system deficit against break-even; \$3.1B against 3% operating margin
- Vermont hospitals requested \$285M in budget increases for FY25 vs. FY24 — suggesting the conservative scenario is already understated
- UVM Health Network alone faces \$1.5B cumulative deficit over 5 years under the high-expense scenario
- Average silver exchange plan premium reached \$985 in 2024 — a 108% increase in 6 years
- Out-of-pocket maximums more than doubled in 5 years
- Hospital charges grew 38% since 2018; median household income grew only 22%

These are not projections of modest stress. They are projections of systemic collapse without intervention. The entire Vermont legislative reform agenda from 2022 to 2025 is a response to this specific data.

The Demographic Trap

Vermont's population dynamics create what Oliver Wyman called a demographic trap — a self-reinforcing cycle from which conventional market responses provide no escape. Vermont's population is aging and shrinking simultaneously: the 65+ population is projected to increase 57% by 2040, rising from 21.7% in 2020 to exceed 30% of total population. The working-age population (20-64) will decline 13% by 2040, from 367,000 to 318,000. By the mid-2030s, Vermont will have nearly as many residents over 65 as under 20.

The consequences cascade in multiple directions at once. Aging drives higher utilization of complex, expensive services: cancer, heart disease, stroke, dementia, and frailty-related hospitalizations all increase with age. A rising 65+ population means a rising proportion of Medicare and Medicaid patients — payers that reimburse below cost — at the expense of the commercially insured working-age population that cross-subsidizes hospital operations. The shrinking commercial base makes the cross-subsidy less viable precisely as the demand for it grows. Oliver Wyman was direct: the cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain as demographics shift.

This dynamic operates at the hospital service area level, not just statewide. Oliver Wyman's HSA-level projections showed that every service area except Burlington will experience population decline and aging — with the Northeast Kingdom (Caledonia, Essex, Orleans counties) facing the most severe trajectory. Newport's 65+ population, already at higher proportions than statewide averages, will represent a majority of the HSA's healthcare demand by 2040.

The Interconnected Failure System

Perhaps the most analytically powerful contribution of the Oliver Wyman report is its systems map of how Vermont's healthcare failures interconnect and amplify each other. The map traces how a housing shortage creates a staffing crisis (workers cannot afford to live near the hospitals that need them), how staffing shortages create capacity constraints, how capacity constraints produce ED crowding and delayed discharges, how delayed discharges require expensive travel nursing that drives up costs, how cost increases drive premium increases, how premium increases reduce affordability, how reduced affordability produces avoidable ED visits from patients who delayed primary care, completing the cycle.

External forces compound these internal dynamics: inflation drives labor and supply costs beyond what hospital budgets can absorb; an aging population shifts the payer mix toward lower-reimbursing Medicare and Medicaid; the cumbersome Certificate of Need process slows capital investment; licensure complexity impedes provider recruitment; and out-of-state care migration reduces procedure volumes, further undermining the clinical volume needed to sustain specialist expertise.

The systems map has a direct implication for policy design: interventions that address only one node of this system will be absorbed by the others. A workforce recruitment program without affordable housing produces recruits who cannot stay. A primary care access expansion without transportation infrastructure does not reach rural patients. Reference-based pricing without global budgets controls unit prices but allows volume to expand. Oliver Wyman's three imperatives are formulated precisely to address the highest-leverage nodes simultaneously, not sequentially.

Healthcare Equity Gaps

Oliver Wyman documented equity challenges across three categories that map directly onto the HTR equity pillar's disparity root cause taxonomy. Gaps in culturally competent care were the most frequently cited equity challenge in community meetings — particularly for patients who do not speak English as a first language, LGBTQ+ patients, patients with disabilities, and patients in rural communities with cultural mistrust of mainstream healthcare institutions. Vermont's foreign-born population, while lower than the national average at 4.5%, is concentrated in Burlington and surrounding communities where language access remains inadequate relative to need.

Gaps in workforce culture and psychological safety were the second category — not just about patients but about providers. Community meeting participants described weight bias, stigma, and a lack of diversity in clinical leadership that affects both care quality and staff retention. Keeping healthcare staff is a function of cost of living, inflation, and affordable housing, multiple stakeholders noted — connecting the workforce equity problem directly to the SDOH infrastructure problem.

Lack of coordination between healthcare and community services was the third category — the failure to close referral loops between clinical care and housing, transportation, food security, and behavioral health. Multiple meeting participants noted that EMR systems do not talk to each other, that patients must navigate 31 EMS agencies for transfers, and that no centralized system guides patients through the coverage and financial assistance they are entitled to.

The Perception vs. Reality Problem

One of Oliver Wyman's most valuable contributions is a systematic correction of widely held misconceptions about Vermont's healthcare infrastructure — misconceptions that, if left uncorrected, produce policy responses calibrated to a system that does not exist. The following table reproduces Oliver Wyman's analysis:

Common perception	What the data actually shows
Vermont has an 'All Payer' model	The 'All Payer' system does not include FFS Medicare or approximately 40% of commercially insured Vermonters. It covers a minority of total healthcare spending.

Vermont has a statewide ACO (OneCare)	OneCare participation is voluntary and does not include several hospitals or many community-based providers. OneCare has been unable to provide the information needed to guide patient care and anticipate needs.
Vermont has a functioning State HIE (VITL)	Participation in VITL is voluntary and many community-based providers are not included. VITL lacks pharmacy claims data and is not viewed as user-friendly by providers.
Vermont does not have enough primary care providers	If primary care providers were supported to see 3 patients per hour, there are enough for well into the future. HRSA recognizes no Health Profession Shortage Areas in Vermont. The problem is productivity and practice model, not pure headcount.

These corrections matter for the book's argument because they explain why Vermont's reform agenda is more complex than it appears from the outside. A state that appears to have all-payer payment reform, an ACO, and a health information exchange actually has voluntary, partial implementations of each — generating the appearance of a transformed system while the underlying fee-for-service, fragmented, data-poor reality persists. Acts 167 and 68 are, in part, a legislative response to this gap between perception and reality.

Act 167 (2022): The Diagnostic Mandate

Vermont passed Act 167 in the immediate aftermath of the COVID-19 pandemic, when the financial fragility of Vermont's hospital system had become impossible to ignore. The act had three core directives: commission a comprehensive statewide hospital systems analysis; direct AHS and GMCB to develop a federal multi-payer payment model proposal; and mandate design work on hospital global budgets. The three directives are analytically linked — you cannot design a credible global budget without understanding each hospital's actual cost structure and community role, and you cannot negotiate a federal model without a clear picture of total cost of care and where it diverges from benchmarks.

The Oliver Wyman Process: A Template for System-Level Reform

GMCB contracted with Oliver Wyman at a fixed price of \$1,050,000 for work running from August 2023 through September 2024. The scale of the engagement was unusual for a state of Vermont's size: over 230 meetings across all 14 Hospital Service Areas, involving more than 3,100 participants from over 100 organizations. Meeting types included 57 sessions with hospital leadership and boards, 45 sessions with state partners, 50 public HSA-level community meetings averaging 68 participants each, 35 provider meetings, and 15 sessions specifically designed to reach diverse populations whose voices are typically absent from healthcare planning processes.

The political economy of this process deserves study because it is a template for system-level reform in any context. Publishing recommendations to close or restructure named services at named community hospitals is not analytically difficult — it is politically treacherous. Communities organize around their local hospitals. Hospital closures generate intense local resistance. The Vermont process managed this by front-loading community engagement before recommendations, establishing the community meetings not as forums for defending the status quo but as inputs to a shared diagnosis. This sequencing — listen first, diagnose second, recommend third — produced findings with enough public legitimacy to survive the inevitable political backlash from hospitals whose specific services were named for restructuring or closure.

The methodology also reflects a design philosophy worth naming explicitly: the report moved from hospital-centered point solutions to systematic redesign; from individual stakeholder consultation to community-driven process; and from post-crisis remedy to pre-planned transformation. These are not rhetorical preferences — they reflect Oliver Wyman's learned lesson from failed hospital reform processes elsewhere, where

solutions designed around individual institutional interests rather than community needs predictably failed to achieve systemic change.

The Five Act 167 Goals as an Accountability Framework

Act 167 established five statutory goals for Vermont's healthcare transformation that have since become the accountability framework for everything that followed: reduce inefficiencies; lower costs; improve health outcomes; reduce health inequities; and increase access to essential services.

These goals matter not as aspirational language but as operational accountability criteria embedded in subsequent reporting and regulatory requirements. AHS's monthly reports to the legislature track progress on these five dimensions. The GMCB's hospital budget review process references them. The Rural Health Transformation Program application explicitly maps each proposed initiative to them. When Act 68's Health Care Delivery Advisory Committee develops the Statewide Strategic Plan, it will be measured against them. This is how statutory goal-setting works when properly designed: not as a preamble, but as a persistent accountability structure that outlives the bill that created it.

Vermont in Practice: Oliver Wyman's Priority Action Lists for 2025

The Oliver Wyman report concluded with explicit priority action lists for each actor in Vermont's governance system — one of the most operationally specific outputs of any state healthcare consulting engagement on record. These lists were published in September 2024; their implementation status one year later is a direct measure of Vermont's reform momentum.

Priority legislative changes for the Vermont General Assembly:

- Remove barriers to building affordable housing for Vermont residents and newcomers
- Approve funding for EMS transformation
- Expand broadband coverage to rural areas
- Review existing AHS agency structure and program list to identify overlaps and efficiency opportunities
- Develop state regulations for Rural Emergency Hospital and free-standing birthing center designations
- Expand professional licensure and practice scope for nurses, EMTs, and pharmacists

Priority transformation programs for AHS to initiate in 2025:

- Regionalize specialty care services across hospitals
- Professionalize and regionalize EMS
- Improve care coordination for heavy utilizers — elderly, mental health, neuro-divergent, and foster care populations
- Develop dual-eligible targeting, care planning, and coordination
- Coordinate and optimize statewide electronic medical record systems

Priority regulatory changes for GMCB:

- Permit no further increases in commercial subsidization for hospital financial shortfalls
- Refrain from licensing any further hospital-based outpatient department units
- Simplify and shorten the Certificate of Need process
- Encourage free-standing diagnostic, ambulatory surgical, and birthing centers
- Begin movement to reference-based pricing, ideally at 200% of Medicare or less for PPS hospitals
- Require all hospitals to use the same accounting agency and method for budget submissions

The alignment between this list and what Act 68 subsequently mandated is not coincidental. The Oliver Wyman recommendations are the intellectual source material for Act 68's operative provisions.

The Oliver Wyman System Redesign Blueprint

The Oliver Wyman report's most detailed section — running from pages 32 through 114 of the full document — provides a three-tier system redesign framework that any healthcare system undertaking structural transformation can apply. The framework organizes recommendations at the state level, the system level, and the individual hospital level, recognizing that each tier requires different interventions executed by different actors on different timelines.

State-Level Recommendations: Infrastructure and Governance

At the state level, Oliver Wyman identified four domains of foundational infrastructure that hospital transformation cannot succeed without, regardless of payment model or clinical redesign. These recommendations are addressed primarily to AHS and the Vermont General Assembly, not to hospitals themselves.

The first domain is governance clarity. Oliver Wyman found that AHS and GMCB are under-resourced for the transformation work they are being asked to lead, that roles between the two agencies need clarification, and that the process requires a dedicated project management office and facilitation support. The specific recommendation to add a Division of Planning and Effectiveness at GMCB — to calculate the impact of service site changes on hospital budgets, monitor access and affordability of community providers, and track quality, access, and equity metrics — is a significant organizational design recommendation that has direct implications for AHS's own restructuring mandate under Act 68.

The second domain is AHS structural alignment. Oliver Wyman recommended that AHS align its agency sub-units with Hospital Service Areas and meet regularly with hospitals and providers in each HSA. It also recommended that AHS fully computerize and integrate its sub-units — an implicit diagnosis that AHS's current organizational structure impedes the kind of coordinated population health management that a transformed system requires. This recommendation is the foundation of the AHS restructuring chapter developed later in this book.

The third domain is state-level investments in Social Determinants of Health: specifically housing, transportation, and broadband. Oliver Wyman was unusually direct about including these as healthcare system recommendations rather than separate social policy recommendations — reflecting the systems analysis that showed housing shortage driving staffing crisis driving cost escalation. The implication is that AHS's healthcare reform mandate cannot be separated from its housing, transportation, and economic development responsibilities. This is a governance challenge as much as a policy challenge.

The fourth domain is administrative simplification: streamlining prior authorization, shortening the CON process, simplifying Medicaid billing, and upgrading IT infrastructure and interoperability. Oliver Wyman documented the direct cost of administrative complexity — a cumbersome CON process prevents cost-effective contracts from being secured in time, and fragmented IT systems mean a provider may need to contact 31 separate EMS agencies to arrange a patient transfer.

System-Level Recommendations: Regionalization

At the system level, the Oliver Wyman blueprint calls for a fundamental redesign of how Vermont's 14 hospitals relate to each other and to the communities they serve. The core concept is regionalization: rather than each hospital attempting to provide a full spectrum of services to its local population — a model that is increasingly unsustainable given population density and physician shortage — Vermont should develop regional Centers of Excellence (COEs) in specific specialties, concentrating volume where it can sustain clinical expertise, while ensuring access through improved transportation and telehealth.

Oliver Wyman developed specific COE assignments for 10 of Vermont's 13 current Hospital Service Areas across 24 specialty categories. The University of Vermont Medical Center received 16 COE designations, covering all major specialties except acute general surgery, mental health emergency department, pediatric psychiatry, and skilled nursing. Southwestern Vermont Medical Center received 9 COE designations. Most community hospitals received between 1 and 6 designations in specialties where they have sufficient volume and expertise to sustain clinical excellence.

Hospital	COE Count	Key Specialties
UVM Medical Center (Burlington)	16	Cancer (complex), neurology, women's health, rehabilitation, most surgical specialties
Southwestern VT Medical Center (Bennington)	9	Cancer surgery (non-complex), geriatric care, orthopedics, psychiatry (adult/adolescent), radiation therapy
Central VT Medical Center (Barre)	5	Geriatric care, infusion therapy, neurology, psychiatry (adult), radiation therapy
Brattleboro Memorial Hospital	6	Acute general surgery, cancer surgery, geriatric care, orthopedics, robotic surgery
Northwestern Medical Center (St. Albans)	4	Cancer surgery (non-complex), infusion therapy, neurology, radiation therapy
Northeastern VT Regional Hospital (St. Johnsbury)	4	Cancer surgery (non-complex), infusion therapy, minimally invasive surgery, radiation therapy
Rutland Regional Medical Center	5	Acute general surgery, geriatric care, minimally invasive surgery, neurology, psychiatry (adult)
Springfield Hospital	2	Memory care, psychiatry (adult)
Copley Hospital (Morrisville)	2	Orthopedics, rheumatology
Mt. Ascutney Hospital (White River Junction)	1	Rehabilitation
Grace Cottage, Gifford, North Country, Porter	TBD	Further discussion required as part of regionalization plan

The COE framework serves multiple goals simultaneously. Concentrating surgical and specialty volumes creates the patient throughput needed to maintain clinical excellence — a surgeon performing 200 procedures a year maintains skills that one performing 20 cannot. Concentrating volumes in financially viable hospitals reduces the total number of facilities that need to sustain high-cost infrastructure. And freeing smaller hospitals from the expectation that they must provide full-spectrum care allows them to redesign their role — becoming hubs for primary care, mental health, geriatric services, and community health rather than trying to maintain services they cannot staff or afford.

The system redesign blueprint also calls for a fundamental shift in the geography of care: move all care possible out of hospitals. Community-based care, primary care, mental health care, and housing capacity should all increase to divert care to lower-cost settings. Home-based care and mobile care programs should expand. Ambulatory surgical units and free-standing diagnostic centers should replace hospital-based equivalents for low-risk procedures. The savings from this shift — estimated at over \$300 million in indirect savings over five years — come from delivering the same clinical value at a fraction of the cost of hospital-based provision.

Hospital-Level Recommendations: Repurposing or Closure

At the individual hospital level, Oliver Wyman provided the most politically sensitive recommendations in the report: several hospitals are at risk of closing their inpatient beds and should consider repurposing their facilities and clinical staff through one of several designated models. The three primary repurposing options are the Rural

Emergency Hospital designation (providing 24/7 emergency services and observation without inpatient beds), the Community Ambulatory Care Center (outpatient and primary care services without inpatient capacity), and the Care at Home support program (community-based services without a hospital facility).

Oliver Wyman was explicit that these recommendations are not about abandoning communities — they are about matching the level and configuration of facility to actual community health need and financial sustainability. A community that cannot sustain a full inpatient hospital but has reliable access to a Regional Specialty Center via improved EMS and transportation is better served than one that maintains an understaffed, financially distressed inpatient facility that cannot reliably staff its emergency department or maintain physician expertise across specialties.

The report also identified specific operational improvements for hospitals that will remain inpatient facilities: UVMHC was specifically called out to examine its overhead and administrative costs, particularly the proportion of providers supporting non-patient-care activities. Reducing exceedingly high administrative costs at some hospitals was identified as one of the three primary sources of direct savings — contributing to the more than \$100 million in direct savings projected from administrative consolidation and shared services.

Vermont in Practice: The \$400M Transformation Savings Estimate

Oliver Wyman projected that full implementation of the hospital transformation recommendations would yield more than \$400 million in direct savings over five years — independent of savings from payment reform activities like AHEAD. The savings sources break down as follows:

Sources of direct savings (more than \$100M):

- Close inpatient facilities with unsustainable financials, whose deficits would otherwise require escalating cash infusions
- Reduce hospital administrative costs, which are exceedingly high at some facilities and have been driving budget increases system-wide
- Reduce costs through synergies — shared services and staff across individual hospitals

Sources of indirect system savings (more than \$300M):

- Deliver same procedures in ambulatory surgical units and outpatient settings, where low-risk procedures can be performed at substantially lower cost
- Expand access to primary care settings to manage patient needs, reducing preventable emergency department visits
- Address social determinants of health, reducing inpatient stays and ED visits that are avoidable if housing and transportation needs are met upstream

These savings can be reallocated to improve Vermont's healthcare system outside of hospital-based care: investing in community-based care (primary care, home health), investing in social needs programs (housing, mental health), and bending the trend of rising health insurance premiums.

The \$400M figure is significant in context: Vermont's Rural Health Transformation Program award of \$195M over five years represents less than half of the projected savings from structural transformation alone. The capital investment is smaller than the structural problem it is meant to help solve — which is why AHS has been clear that the RHT funding is a tool for executing reform, not a substitute for reform.

Act 68 (2025): The Operational Mandate

Signed by Governor Scott on June 12, 2025, Act 68 is the legislative operationalization of the Oliver Wyman diagnosis. The alignment between Oliver Wyman's priority recommendations and Act 68's operative provisions is direct and traceable: Oliver Wyman recommended beginning movement to reference-based pricing at 200% of Medicare or less; Act 68 mandated it by hospital fiscal year 2027. Oliver Wyman recommended moving to global budgets when conditions for success are met; Act 68

required them for non-critical access hospitals by FY2028 and all hospitals by FY2030. Oliver Wyman recommended that GMCB add a Division of Planning and Effectiveness; Act 68 authorized three new permanent classified positions at GMCB. Oliver Wyman recommended reviewing AHS agency structure and program list; Act 68 directed AHS to develop a Statewide Health Care Delivery Strategic Plan by December 2028.

The act's purpose statement is worth reading in full because it encodes the Oliver Wyman framework into law. The General Assembly declared the purpose of Act 68 to be transformation of and structural changes to Vermont's health care system, advancing five goals: improvements in health outcomes and reducing access disparities; an integrated care system with robust coordination and increased investment in primary care, home health, and long-term care; stabilizing providers and controlling costs beginning with reference-based pricing and continuing to global hospital budgets; evaluating progress through standardized accountability metrics; and establishing a system that attracts and retains high-quality health care professionals. Every one of these goals is a direct response to a finding in the Oliver Wyman report.

The Reference-Based Pricing Architecture

Act 68's RBP provisions represent a significant departure from the voluntary, incentive-based payment reform approach that dominated American health policy for the previous fifteen years. The act does not invite hospitals to experiment with alternative pricing — it directs GMCB to establish, by rule, maximum amounts that hospitals shall accept as payment in full, not later than hospital fiscal year 2027. The language is mandatory and statewide.

The underlying price problem that RBP addresses is more severe than most national discussions of hospital pricing acknowledge. GMCB's February 2026 report documented that Vermont hospitals charge commercial payers approximately 250-300% of Medicare rates on average — but this average conceals extraordinary variation. Outpatient imaging at some Vermont hospitals reaches 715-944% of Medicare rates. Inpatient mental health services at the same hospitals range from 123% to 409% of Medicare. UVMHC outpatient charges specifically averaged 417% of Medicare rates in 2022 — among the highest in the nation. Hospitals are not clustered near a rational break-even point of 136% of Medicare; they are distributed across a range from roughly profitable to wildly extractive, with no relationship between price and quality.

The design principles embedded in Act 68's RBP statute reflect the cross-state evidence base. Prices must be based on a percentage of Medicare reimbursement — establishing a transparent, nationally comparable anchor. Consideration must be given to each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. And hospitals are explicitly prohibited from charging patients or insurers more than the established reference-based price, closing billing workarounds that undermined earlier price transparency experiments.

State / Program	Key design features and outcomes
Vermont Act 68	Mandatory statewide maximum allowable payments through GMCB rate-setting authority. Methodology set by rule FY2027, effective FY2028. RBP then reviewed annually as part of hospital budget process.
Oregon (2019)	State employee plan only. 200% in-network / 185% out-of-network cap. Exemptions for rural and critical access hospitals. Savings exceeded \$107.5M in first 27 months. All 24 affected hospitals remained in-network.
Montana (2016)	State employee plan. Direct contracting at percentage of Medicare. Initial \$48M in savings but politically fragile without statutory mandate — illustrating Vermont's choice to legislate rather than demonstrate.
Washington (2021)	Public option ('Cascade Care'). Hospital reimbursement capped at 160% of Medicare for participating networks. Voluntary participation, expanding to state employee plans.

Maryland (2014–present)	All-payer system with fixed annual revenue for hospitals based on standardized prices and volume expectations. Includes Medicare waiver. Strong incentives for population health management. The closest model to Vermont's Act 68 trajectory.
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The Global Budget Mandate

Act 68 requires GMCB to establish global hospital budgets for non-critical access hospitals by hospital fiscal year 2028, and for all Vermont hospitals by 2030. This statutory obligation makes Vermont the first U.S. state outside Maryland on a legislatively mandated path to a comprehensive all-payer global budget system.

Understanding why Vermont needs global budgets in addition to reference-based pricing requires understanding the balloon squeeze problem that Oliver Wyman identified. Reference-based pricing controls unit prices but does not control volume. A hospital facing lower per-service revenue can compensate by delivering more services — scheduling more imaging, performing more procedures, admitting patients who could be managed in outpatient settings. Total hospital spending equals price times volume; controlling only price leaves volume as a release valve. Global budgets close this loophole by capping total hospital revenue regardless of service volume, fundamentally changing the hospital's financial incentive from maximizing throughput to maximizing population health per dollar of revenue.

Vermont has attempted global budget design four times in the past decade: the GMCB net patient revenue cap in 2012, the Rutland Regional pilot proposed in 2014 (never implemented), the Vermont All-Payer ACO Model from 2017 to 2025, and the current Act 167/Act 68 mandate. Each prior attempt foundered on the same obstacle: voluntary participation. When payers can opt out of a global budget arrangement, the highest-cost, most commercially valuable services migrate out of the capped system, leaving lower-reimbursing services behind and making the global budget financially unworkable. Act 68 eliminates the voluntary option. This is the critical design difference between Vermont's current trajectory and everything that preceded it.

The Statewide Health Care Delivery Strategic Plan and AHS Restructuring

The most consequential long-term provision of Act 68 is the requirement that AHS develop a Statewide Health Care Delivery Strategic Plan, to be delivered to the legislature by December 2028 and updated every three years thereafter. This plan is the statutory vehicle for answering the foundational question that underlies all of Vermont's reform work: what should Vermont's healthcare delivery system actually look like?

Act 68 established the Health Care Delivery Advisory Committee to support the plan's development, with responsibilities including setting affordability benchmarks, monitoring system performance, advising on the continuous evaluation process for the healthcare delivery system, and working with AHS to develop and maintain the strategic plan. The committee's mandate also requires incorporating recommendations from the Vermont Steering Committee for Comprehensive Primary Health Care — ensuring that primary care's central role in the redesigned system is structurally embedded in the planning process.

Oliver Wyman's state-level recommendations are the most detailed public blueprint available for what this strategic plan should address. The four domains Oliver Wyman identified — governance clarity, AHS structural alignment with HSAs, state-level SDOH investments, and administrative simplification — constitute the operational dimensions of the strategic plan. The AHS restructuring chapter later in this book develops a full six-pillar framework for how AHS should approach this planning mandate, drawing directly on both the Oliver Wyman recommendations and the Act 68 statutory requirements.

For healthcare leaders, policymakers, and organizations doing business with Vermont's system, the following statutory deadlines define the transformation calendar:

FY2026 (underway as of this writing):

- GMCB hospital budget guidance includes negative 1% commercial rate growth benchmark — the first negative benchmark in Vermont history
- AHS hospital transformation grants of \$2M under Act 68; all 14 hospitals developing transformation plans with RHRC support
- Vermont Medicaid global budget for hospitals begins January 2026 under AHEAD Cohort 2 State Agreement
- Act 55 outpatient drug cap at 120% of ASP effective April 2025, generating estimated \$135M in annual savings

FY2027 (effective October 2026):

- Reference-based pricing for hospital services implemented — maximum amounts hospitals can charge commercial payers, set by GMCB rule
- GMCB publishes hospital price transparency dashboard
- RBP vendor selection and methodology finalized; rulemaking completed

FY2028 (effective October 2027):

- Global hospital budgets established for non-critical access hospitals
- AHS Statewide Health Care Delivery Strategic Plan delivered to legislature by December 2028
- AHEAD Model Cohort 2 full operation; Medicare FFS hospital global budgets active
- CMMI transformation grants of \$1.2M available through AHEAD Cooperative Agreement Year 3

FY2030 (effective October 2029):

- Global hospital budgets for all Vermont hospitals, including critical access hospitals

These deadlines are not aspirational. They are statutory obligations. Organizations planning capital investments, contract negotiations, workforce strategies, or technology deployments that interact with Vermont's hospital system must treat these dates as certainties, not contingencies.

The Rural Health Transformation Program: Capital for an Operational Strategy

In November 2025, AHS submitted Vermont's application to the federal Rural Health Transformation Program — a \$50 billion national fund created by the One Big Beautiful Bill Act of 2025, with \$10 billion available annually from 2026 through 2030. Vermont was awarded \$195 million in December 2025, nearly double what the state had expected to receive and among the highest per-capita awards nationally.

The RHT application is itself a teaching document for healthcare transformation practice, because it is structured entirely around the Act 167 and Act 68 goals. Vermont's vision, stated explicitly in the application, is to ensure that rural residents have access to the right care, at the right time, in the right place, at an affordable cost — a direct echo of the AHS health care reform vision statement. The application's three strategic goals map onto Oliver Wyman's three imperatives: strengthening hospital sustainability and primary care (Move all care possible out of hospitals); building workforce pipeline and retention capacity (Build housing and fix the conditions that drive workforce shortage); and leveraging innovative strategies to reduce costs (Pay with reference-based pricing and move to global budgets).

AHS Secretary Samuelson's framing of the award is the correct analytical frame: the \$195 million is not Vermont's healthcare reform plan. It is capital to execute a plan that already exists. The funding is explicitly one-time money, and AHS has stated publicly that it must be treated as a supplement to ongoing reform work, not a lifeline. The distinction between investment capital and reform strategy is one of the most persistently confused in health policy. States that treat federal grants as reform strategies — spending the money on programs without addressing underlying structural

problems — predictably return to crisis when the grant period ends. The Vermont approach, which explicitly aligns every RHT initiative to the statutory Act 167 and Act 68 goals, is a model of how capital investments should be positioned within a structural reform agenda.

The specific initiatives funded through Vermont's RHT award include telehealth expansion for specialty care access in rural hospitals, workforce recruitment and retention programs with five-year service obligations, hospital analytics infrastructure to support regional transformation planning, dialysis capacity development for nursing home residents, and CCBHC expansion to certify five additional community behavioral health clinic entities by July 2026. Each of these investments addresses a specific node in Oliver Wyman's systems failure map — and each is designed to reduce ongoing structural costs rather than subsidize their continuation.

Cross-State Lessons: What Vermont's Model Generalizes

Vermont's reform architecture is instructive because it addresses structural problems present, in varying degrees of severity, across rural America. The analytical framework Oliver Wyman applied, and the legislative mechanisms Vermont enacted in response, generalize well to any state or regional system facing demographic aging, hospital financial fragility, workforce shortage, and the failure of voluntary payment reform to achieve systemic change.

Five generalizable lessons emerge from the Vermont case.

First, diagnosis precedes legislation. Vermont's Act 68 is as effective as it is because it was preceded by Act 167's comprehensive, public, community-grounded diagnostic process. Legislatures that skip the diagnostic phase and proceed directly to structural mandates produce policies that are technically sound but politically unsustainable. Vermont's sequence — diagnose publicly, build shared understanding, then mandate — is a process design that other states should study.

Second, voluntary reform achieves voluntary compliance. Vermont's decade-long experience with voluntary global budget arrangements — the All-Payer ACO Model, OneCare participation, VITL connectivity — produced partial, fragile outcomes precisely because voluntary participation allows the highest-cost actors to opt out. The Oliver Wyman report documented this plainly in its perception-versus-reality analysis. Act 68's mandatory architecture is a direct response to that experience.

Third, price control without volume control is insufficient. Reference-based pricing is necessary but not sufficient for controlling total hospital spending. The Act 68 sequence — RBP first, then global budgets — reflects the analytical understanding that price and volume must both be addressed. Oregon's RBP experience, Montana's experience, and Maryland's global budget model all confirm this.

Fourth, housing and transportation are healthcare infrastructure. Oliver Wyman's finding that housing shortage drives staffing crisis drives cost escalation is not a soft social observation — it is a hard systems analysis. Healthcare reform strategies that treat housing and transportation as separate social policy domains rather than healthcare infrastructure will fail to achieve the workforce stability and access improvements they require.

Fifth, the AHS restructuring is the strategic plan. Oliver Wyman's explicit recommendation that AHS align its sub-units with Hospital Service Areas, add a Division of Planning and Effectiveness, and restructure to better coordinate health and social service needs at the community level is the organizational design specification for what Act 68's Statewide Strategic Plan must produce. The book develops this specification fully in the AHS Restructuring chapter that follows.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Reform cascade	A legislatively structured process in which each reform intervention creates the institutional capacity and political legitimacy for the next. Vermont's Act 167 → Act 51 → Act 68 sequence is the exemplar.
Platform for Change	Oliver Wyman's term for the five interconnected challenge domains — decreasing affordability, deteriorating sustainability, demographic trap, interconnected failure system, and equity gaps — that collectively make structural transformation unavoidable for Vermont.
The three imperatives	Oliver Wyman's highest-level summary of what Vermont must do: build housing and fix transportation; pay PPS hospitals with reference-based pricing and move to global budgets; move all care possible out of hospitals.
Center of Excellence (COE)	A hospital or facility designated to concentrate volume and expertise in a specific specialty, receiving referrals from other hospitals and ensuring that patient volumes are sufficient to maintain clinical quality. Vermont's regionalization plan designates COEs across 24 specialty categories at 10 of 14 current Hospital Service Areas.
Balloon squeeze problem	The phenomenon in which controlling hospital prices without controlling volume allows hospitals to compensate for lower per-service revenue by delivering more services. Reference-based pricing controls price but not volume; global budgets are required to address both simultaneously.
Statewide Health Care Delivery Strategic Plan	The plan AHS is required to deliver to the Vermont legislature by December 2028 and update every three years, establishing the vision, goals, affordability benchmarks, and accountability metrics for Vermont's transformed health care delivery system. Established by Act 68.
Hospital Service Area (HSA)	A geographic area defined around a hospital and the population it primarily serves. Vermont has 14 HSAs corresponding to its 14 hospitals. The HSA is the fundamental unit of analysis for Vermont's regionalization and transformation planning.
Rural Emergency Hospital (REH)	A federal designation allowing a hospital to provide 24/7 emergency services and observation care without maintaining inpatient beds — an option for Vermont hospitals that cannot sustain full inpatient capacity but need to maintain emergency access for their communities.

Sources: Act 167 of 2022 (Vermont); Act 51 of 2023 (Vermont); Act 68 of 2025 (Vermont); Oliver Wyman Healthcare and Life Sciences Group, Act 167 Community Engagement: Recommendations, September 2024 (revised October 2024 and January 2025); Green Mountain Care Board, Act 68 Update on Hospital Reference-Based Pricing and Global Budgets, February 17, 2026; Vermont Agency of Human Services, Vermont Rural Health Transformation Program Application, November 3, 2025; Vermont Agency of Human Services / Brendan Krause, Vermont's Health Care Reform Efforts, presented to Vermont House Health Care Committee, January 31, 2025; Green Mountain Care Board, 2024 Annual Report, January 15, 2025.